



The \$35 Cap and the People It Misses

Teacher copy (everything in the student copy, plus answers and teaching notes)

The article, marked for rhetorical devices (one color throughout):

1 Thirty-five dollars is the number that did the work. Under the Inflation Reduction Act, the Centers for Medicare and Medicaid Services capped out-of-pocket insulin at \$35 a month for Medicare Part D beginning January 1, 2023, and for Part B that July. About 1.5 million Medicare beneficiaries who use insulin stood to benefit, with annual savings the federal government put at \$734 million in Part D and \$27 million in Part B. The figure is clean, the relief is real, and it stops where Medicare stops.

2 The cap reaches Part D and Part B beneficiaries. It does not reach the uninsured. It does not reach most people on private plans, and it does not reach those covered by Medicaid alone. [Anaphora] Twenty-nine states and the District of Columbia have written their own \$35 ceilings into state-regulated health plans, but those, too, hold only inside their own borders and their own program rules. The median monthly out-of-pocket price runs about \$40 for someone with private insurance and about \$100 for someone with none.

3 The list prices have come down a long way. [Telegraphic sentence] Eli Lilly cut several of its insulins by more than 70 percent in 2023; Novo Nordisk followed with reductions of 75 percent on Fiasp and 72 percent on Tresiba, effective January 1, 2026. New suppliers arrived alongside them. Civica Rx launched a long-acting glargine biosimilar this January at a \$45 wholesale price and a recommended ceiling of \$55 for a box of five pens, and California's CalRx put a state-branded glargine on shelves at \$11 a pen. Civica is a nonprofit; CalRx is a state. The companies that already controlled the market did not build either one.

4 That market is narrow. Novo Nordisk, Eli Lilly, and Sanofi together hold more than 90 percent of the global insulin supply and are the only firms selling insulin to patients in the United States. [Logos] The barriers to a fourth entrant are documented and dull: an interchangeability approval that no insulin product has ever received, trade-secret protection on the manufacturing, patents on the delivery devices, and development costs running \$100 million to \$250 million over seven or eight years. Biosimilars like Semglee and Rezvoglar now sell for 65 to 85 percent less than the brand analogs, which tells you how much room the old prices held.

5 So the headline numbers and the survey numbers do not agree. [Juxtaposition] In October 2025 the American Diabetes Association found insulin still unaffordable for almost 30 percent of people with diabetes. One in six insulin users rations the drug because of cost; among uninsured respondents, 39 percent said they had rationed in the past six months. Rationing means taking less than the prescribed dose, or skipping it, to make a vial last. Manufacturer assistance programs offer free insulin to some who earn up to 400 percent of the federal poverty level, roughly \$60,240 for an individual in 2026, though the enrollment is restrictive and hard to navigate.

6 In March 2026, senators Warnock, Shaheen, Kennedy, and Collins introduced the INSULIN Act, which would extend the \$35 monthly cap to people with private insurance and set up a pilot grant program for the

uninsured. It has not passed. For now the \$35 figure means one thing on Medicare, something narrower in 29 states, and nothing at all for the person paying cash at the counter.

Devices, and why they are here:

Anaphora (paragraph 2): “The cap reaches Part D and Part B beneficiaries. It does not reach the uninsured. It does not reach most people on private plans, and it does not reach those covered by Medicaid alone.”

The repeated “It does not reach” at the head of successive clauses lets the author enumerate who the cap excludes without editorial comment; the structure itself carries the limitation, keeping the Ledger’s flat tone while the list of the uncovered accumulates weight.

Telegraphic sentence (paragraph 3): “The list prices have come down a long way.”

The clipped opener resets the paragraph and states the price-drop fact bluntly before the qualifying detail arrives; the short flat line gives the Ledger a plain declarative anchor against the longer attributed sentences that follow.

Logos (paragraph 4): “Novo Nordisk, Eli Lilly, and Sanofi together hold more than 90 percent of the global insulin supply and are the only firms selling insulin to patients in the United States.”

The market-concentration claim is built entirely from a named figure and named firms, letting the documented record, not the writer’s adjectives, establish why competition is thin; it is the register’s central appeal, credibility borrowed from the cited fact.

Juxtaposition (paragraph 5): “So the headline numbers and the survey numbers do not agree.”

Setting the price-drop “headline numbers” directly beside the “survey numbers” of persistent rationing sharpens the gap between announced relief and lived cost without the author asserting a contradiction; the two facts are placed side by side and left to register.

AP-style multiple choice:

1. In the second paragraph (“The cap reaches Part D and Part B beneficiaries...”), the repeated construction “It does not reach” primarily serves to
 - A) concede that the cap has failed and should be repealed by Congress
 - B) enumerate the groups the cap excludes so the limitation registers through accumulation rather than assertion
 - C) prove that state-level caps are more effective than the federal one
 - D) build emotional outrage at the manufacturers who set insulin prices
2. Which of the following best characterizes the writer’s overall position on the 2026 insulin price news?
 - A) Falling list prices have effectively solved insulin affordability in the United States
 - B) The \$35 cap is a failure that has helped almost no one
 - C) The three major manufacturers deserve credit for the recent drop in prices
 - D) The documented relief is genuine but partial, leaving large groups still priced out

Multiple choice answer key:

1. Answer B. The keyed option matches the Ledger's method: the parallel "It does not reach" clauses list the uncovered populations (uninsured, private, Medicaid-only) and let the scope of the exclusion build without the writer editorializing. The outrage-at-manufacturers option imports an emotional aim the flat, attributed paragraph never pursues (right device, wrong effect). The repeal option overstates the text, which reports the cap as real relief that simply stops where Medicare stops, never calling for repeal (over-broad or extreme). The state-caps-are-better option names a comparison the paragraph does not make; it notes states wrote their own ceilings but ranks neither (true but not the function asked).

2. Answer D. The keyed option tracks the whole piece, which credits the cap's real savings and the steep list-price cuts while documenting that nearly 30 percent still cannot afford insulin and the cap misses the uninsured and most private payers. The solved-affordability option inverts the survey evidence the final paragraphs rest on (inverted relationship). The cap-is-a-failure option ignores the stated \$734 million in savings for 1.5 million beneficiaries (over-broad or extreme). The credit-to-manufacturers option contradicts the text, which notes the new low-cost suppliers were a nonprofit and a state, not the incumbent firms (inverted relationship).

Discussion questions:

1. The piece reports both a real \$35 cap with documented savings and an ADA survey showing nearly 30 percent still cannot afford insulin. How does placing these facts next to each other, without the writer naming a contradiction, shape what you conclude?
2. The author attributes almost every claim to a named body, figure, or date (CMS, the ADA, specific senators, exact percentages). What does that habit of attribution do to the writer's credibility, and how would the paragraph on rationing read if the numbers were dropped?

Sample strong discussion responses:

1. Putting the savings figure and the unaffordability survey side by side forces the reader to hold two true things at once: the cap worked for the people it covers, and it left most people untouched. Because the writer never says "but this is hypocrisy" or "the relief is fake," the gap feels like a finding rather than an accusation. The reader supplies the judgment the prose declines to state, which is exactly the Ledger's method. The juxtaposition in the fifth paragraph is load-bearing: drop either number and the piece collapses into either a victory lap or a complaint.
2. The attribution is the credibility. Every load-bearing claim carries a source the reader could in principle check: CMS for the cap dates, the federal government for the \$734 million, the ADA for the October 2025 survey. This is ethos built by restraint rather than assertion; the writer never says "trust me," only "here is who established this." If the rationing paragraph dropped its numbers, "one in six" and "39 percent of the uninsured," it would shrink to an adjective like "many people ration," which the reader has no way to weigh and no reason to believe. The figures are doing the work an emotive word would do in a hotter register.

Writing prompt (Homework or extended in-class, Q3 argument):

The opener documents a \$35 insulin cap that delivered real, measurable savings to the Medicare beneficiaries it covers, while leaving the uninsured, most privately insured, and Medicaid-only patients outside its reach. Some hold that a targeted policy delivering proven relief to one large group is a legitimate and worthwhile step even when it does not reach everyone; others hold that a benefit structured to miss the people in the greatest need, the uninsured who ration most, is a failure of design rather than a partial success. Write an essay in which you defend, challenge, or qualify the claim that a price cap is worth enacting even when it covers only some of the people it could. Use specific evidence from the opener and your own knowledge or experience to support your line of reasoning.

Common misconceptions to watch for:

- Students may read the anaphora in paragraph 2 ("It does not reach...") as an emotional appeal meant to provoke anger. In the Ledger register the repetition is structural: it organizes a flat list of excluded groups and lets scope accumulate, not feeling. Pair this with the device note on anaphora.
- Students may conclude the piece argues the \$35 cap was a mistake or a failure. The text reports genuine savings (\$734 million, 1.5 million beneficiaries) and a real list-price collapse; its position is that the relief is partial, not that it was wrong.
- Students may credit the three big manufacturers for the new low-cost insulins. The opener is explicit that Civica Rx is a nonprofit and CalRx is a state program, and that the incumbent firms built neither; the price competition came from outside the established market.
- Students may treat "rationing" as merely buying less or shopping around. The opener defines it as taking less than the prescribed dose or skipping it to stretch a supply, which is a clinical and safety matter, not a budgeting choice.

AP exam alignment:

This opener teaches students to read an argument that is carried by attributed evidence rather than asserted by the writer, the core of the Ledger register and of AP's Claims and Evidence and Style skills. It models how juxtaposition and anaphora can do analytic work in restrained, source-driven prose, and how a writer can take a clear position (relief is real but partial) without ever stating it as opinion. The Q3 prompt extends the passage's theme into a debatable proposition about targeted versus universal policy.